Wintergate Chambers — Return-to-Work Clearance Source Packet

Fallowbrook Trading Company | Pennsylvania distribution hub 3 | Opening 2 November 2026

Frozen authority and operating material — apply as of 1 September 2026. This packet answers a narrow operational question: what evidence and controls must be reconciled before dispatch may place a worker on a specific assignment. It is not legal advice and does not replace an emergency response.

Working ambiguity. Clinic correspondence commonly calls a worker “cleared” when the clinician has released the worker. Dispatch uses “cleared” when the worker may actually be placed on a shift. These usages are not interchangeable.

Source notes below are deliberately limited to the question each authority answers. Internal controls marked **BINDING INTERNAL CONTROL** are required for this hub; items marked **OPERATING PRACTICE** are recommended workflow aids, not law.

1. Authority boundaries

Source / status	Question it answers	Does not decide
Federal OSHA recordkeeping — 29 CFR Part 1904	Whether a work-related injury or illness is recorded on required logs.	Fitness for a job, restrictions, or whether dispatch may schedule the worker.
Federal driver qualification — 49 CFR 391.41 and 391.43	Whether a worker assigned to a covered commercial motor vehicle has the required current medical qualification / examiner certificate record.	Fitness for unrelated warehouse duties or accommodation.
Federal accommodation framework — ADA, 42 USC 12112	Whether a potential disability-related job modification belongs in an individualized interactive review.	A particular accommodation, a diagnosis disclosure, or an automatic full-duty result.
Pennsylvania overlay — PHRA / employment setting	Pennsylvania nondiscrimination and employment context; this packet applies the hub controls in Pennsylvania.	A separate statewide medical-release rule that overrides federal qualification, safety, or assignment controls.
Wintergate internal attendance policy — BINDING INTERNAL CONTROL	Absence credit, coding, and notification treatment.	Medical fitness or schedulability.
Wintergate assignment / safety controls — BINDING INTERNAL CONTROL	The actual essential and safety-sensitive demands of the proposed job and whether operations can staff it.	A clinical diagnosis or a unilateral waiver of restrictions.

Apply concurrent requirements by subject matter. No source above globally overrides another. The final dispatch answer must preserve every applicable gate.

2. Applicability, records, and evidence

This process covers hub workers returning after illness, injury, treatment, a restriction, a safety concern, an absence requiring return evidence, or a change in assignment. It covers both warehouse/non-driving work and covered commercial driving. Apply federal driver-qualification controls only to a worker proposed for a CMV duty subject to 49 CFR Part 391; do not extend a driver result automatically to ordinary warehouse work.

BINDING INTERNAL CONTROL — intake and privacy. Record worker ID, two approved identifiers (worker ID and date of birth; name alone is insufficient), proposed role/assignment, expected return date/time, driving duty/vehicle coverage, submitted evidence, known functional restrictions, and contact route. Keep clinical records in the restricted occupational-health case file. Dispatch and line management receive only status, permitted duties, effective timing, required restrictions, and review date — never routine diagnosis details. Access is limited to the coordinator, authorized clinical reviewer, HR/accommodation partner, and other role with a documented need to know.

3. Evidence and assignment controls

BINDING INTERNAL CONTROL — before reliance. Match evidence to the worker record using the approved identifiers. A clinical release or qualification document must show: worker match; qualified issuer / applicable credentials; issue or examination date; signature or verifiable authentication; functional capacity statement; all restrictions; and an end date or review date for temporary restrictions. It must be legible, internally consistent, and current for the proposed return. A driver record for covered CMV duty must additionally show the current required medical qualification / examiner certificate record and no known disqualifying restriction.

Obtain the actual proposed assignment from the hub manager before evaluating a restriction. The assignment profile identifies essential demands and safety-sensitive elements, including lifting/pushing, standing/walking, climbing or work at height, powered-equipment operation, and covered CMV driving. A generic “fit for work” statement is not a substitute for the comparison.

BINDING INTERNAL CONTROL — evidence exceptions. Incomplete, illegible, ambiguous, expired, or internally inconsistent information is not inferred in the worker’s favor. Preserve the item, state the precise missing or conflicting point, and request clarification from the issuer through the worker or from the authorized clinical reviewer. Conflicting valid documents are held for the Occupational Health Clinical Reviewer; the coordinator does not select one. A material change in symptoms, condition, restrictions, or proposed assignment after release pauses the prior scheduling answer and requires reassessment.

BINDING INTERNAL CONTROL — restrictions. Do not demand “fully healed,” a zero-restriction note, or diagnosis. Compare functional limits with actual assignment demands. The coordinator may not reject, waive, or create modified work. Send a viable restricted-duty question to the Hub Operations Manager and HR/Accommodation Partner for authorized assignment/interactive review; preserve the restriction pending their decision. Temporary approval must have a dated review or end date.

4. Outcome status vocabulary (use these labels exactly)

Status	Meaning and scheduling consequence
Scheduled — Unrestricted	All applicable gates are satisfied for the named assignment. Dispatch may schedule the worker for that assignment from the documented effective time.
Scheduled — Approved Modification	Authorized assignment/accommodation review approved the stated modification. Dispatch may schedule only the stated duties and restrictions through the dated review/end point.
Pending — Safety/Evidence Hold	A safety, evidence, conflict, reassessment, capacity, or required review gate remains unresolved. Do not place the worker on the schedule; reconsider when the named issue is resolved.
Driving Not Qualified — Non-driving Review	The worker may not be scheduled for covered CMV driving because qualification is missing, expired, invalid, or disqualifying. Hub Operations reviews eligibility for non-driving duties separately.
Emergency Hold — Do Not Schedule	Acute warning signs invoke the emergency route. No routine clearance or scheduling resumes until that route permits it.

5. Acute-warning-sign exception — stop routine processing

Trigger: even with a facially valid release, stop the ordinary workflow if the worker presents or reports chest pain/pressure; difficulty breathing; fainting; new confusion; face droop, arm weakness, or speech difficulty; uncontrolled bleeding; seizure; or a new severe allergic reaction.

Emergency route — BINDING INTERNAL CONTROL: assign **Emergency Hold — Do Not Schedule**; call 911 / the site emergency response number and notify site security and the Hub Operations Manager. Do not direct the worker to drive or use ordinary clearance messaging. Document only necessary observations, time, and handoff. Notify the Occupational Health Clinical Reviewer after emergency services are engaged. A prior release does not authorize work while the concern remains unresolved. Routine processing resumes only after emergency responders release the immediate event and the authorized clinical reviewer confirms that normal evidence/assignment review may restart.

6. Timing, appointments, and one-shift objective

BINDING INTERNAL CONTROL — operating window. The desk operates 07:00–15:30 on each opening-week operating day. Intake appointments and complete submissions received by 13:00 can be same-shift eligible; an appointment unavailable until after 13:00 or a complete submission after 13:00 is next-operating-shift eligible. The “one shift” interval starts when a coordinator receives a complete intake packet, confirms worker identity and proposed assignment, and detects no acute-warning trigger. It ends when an applicable outcome status and its effective timing are documented and the permitted scheduling information is sent to dispatch. It does not start for a missing/invalid item and pauses while awaiting an external issuer, emergency route, clinical decision, qualification decision, accommodation/assignment decision, or capacity needed to process it.

Service objective — BINDING INTERNAL CONTROL. For the opening-week forecast, achieve at least 95% same-shift eligible workload completion on *each* operating date and finish 6 November with no workload backlog. This is a planning objective, not a promise for an individual case. It remains conditional on appointment availability, a complete before-cutoff packet, no emergency or changed circumstance, needed reviewers/issuers responding, and available capacity.

7. Opening-week forecast and modeling inputs

Operating date	Unrestricted	Restricted	Driver review	Evidence clarification	Total arrivals
02 Nov 2026	20 (18 eligible)	8 (7 eligible)	7 (6 eligible)	5 (4 eligible)	40
03 Nov 2026	18 (16 eligible)	7 (6 eligible)	6 (5 eligible)	4 (3 eligible)	35
04 Nov 2026	20 (18 eligible)	8 (7 eligible)	7 (6 eligible)	5 (4 eligible)	40
05 Nov 2026	19 (17 eligible)	8 (7 eligible)	7 (6 eligible)	4 (3 eligible)	38
06 Nov 2026	17 (15 eligible)	6 (5 eligible)	6 (5 eligible)	4 (3 eligible)	33

Eligible means complete before the 13:00 cutoff; other arrivals still consume workload and roll into backlog. Forecasted case types are workload categories, not an outcome determination. Workload factors: unrestricted 0.25 hour/case; restricted 0.75 hour/case; driver review 0.50 hour/case; evidence clarification 0.40 hour/case. First-day opening backlog: **4.25 workload hours**. Daily completion equals the lesser of available workload and available capacity; eligible workload is processed before backlog and after-cutoff arrivals for the same-shift metric.

8. Supplied coverage options and decision standard

Option	Staffing / capacity inputs	Cost inputs
Baseline desk coverage	2 coordinators × 8 scheduled hours/day. Subtract 2 nonproductive hours/day; subtract an additional 2 hub-orientation hours on 02 Nov.	$32 per scheduled coordinator hour + $500 fixed launch cost.
Enhanced launch coverage	3 coordinators × 8 scheduled hours/day. Subtract 3 nonproductive hours/day; subtract an additional 2 hub-orientation hours on 02 Nov.	$32 per scheduled coordinator hour + $900 fixed launch cost.

Capacity is gross scheduled staff-hours less the stated nonproductive allowance and the date-specific orientation constraint. Do not add costs or capacity factors not shown above. Select the **least-cost supplied option that meets the service objective**.

9. Escalation, communication, continuity, and quality controls

Situation	Owner / handoff	Schedule while awaiting / timing
Missing, unclear, or conflicting clinical evidence	Occupational Health Clinical Reviewer; send record match, assignment profile, document, and precise discrepancy.	Pending — Safety/Evidence Hold; response target same shift if complete before 13:00, otherwise next operating shift.
Covered CMV qualification missing/expired/invalid	Fleet DQ Custodian; send worker ID, proposed CMV duty, DQ record, and expiry/defect.	Driving Not Qualified — Non-driving Review; no covered driving. Non-driving review starts with Hub Operations same shift when feasible.
Restriction / potential accommodation	Hub Operations Manager and HR/Accommodation Partner; send functional limits, duration, and assignment demands — not routine diagnosis.	Pending — Safety/Evidence Hold until authorized result; acknowledgement by next operating shift.
Capacity / appointment constraint	Clearance Desk Supervisor and Hub Operations Manager; send queue age, eligible workload, and proposed timing.	Pending — Safety/Evidence Hold or next-shift timing; no one-shift guarantee.
Acute warning sign	911/site emergency, security, Hub Operations, then Occupational Health Clinical Reviewer; send necessary observation/time/handoff.	Emergency Hold — Do Not Schedule; only the emergency route may permit resumption.

BINDING INTERNAL CONTROL — case record. Retain evidence reviewed, identity match, assignment profile, gate results and decision basis, status, responsible decision maker, receipt/decision/communication timestamps, communications, and dated review/end point. Worker and operations notices state status, permitted duties, effective time, scheduling restrictions, and next review only. Attendance/payroll receives attendance routing separately; attendance treatment never creates schedulability. OSHA logging is referred to safety/recordkeeping separately and never decides fitness.

Reconsideration and downtime. A worker or referring role may submit corrected evidence or request reconsideration through the coordinator. Link it to the existing case, preserve the prior record, route it to the responsible reviewer, and document the new decision and rationale. During a system or communication outage, use the controlled paper downtime log with worker ID plus second identifier, receipt/decision timestamps, restricted storage, and named recipient confirmation; reconcile it to the case file when service returns. Do not make an informal clearance by chat or verbal relay.

OPERATING PRACTICE — supervisor check. The Clearance Desk Supervisor reviews the queue at opening and 14:30 daily, sampling completed files and every pending case for missing gates, overdue reviews, status/schedule mismatch, and unsupported decision basis. Correct before dispatch use where possible; document correction and retrain/escalate recurring errors.

Source notes: OSHA 29 CFR Part 1904; FMCSA 49 CFR 391.41 and 391.43; ADA 42 USC 12112; Pennsylvania Human Relations Act / employment context; Wintergate internal controls. This task-specific frozen summary is limited and should not be treated as a complete legal compendium.